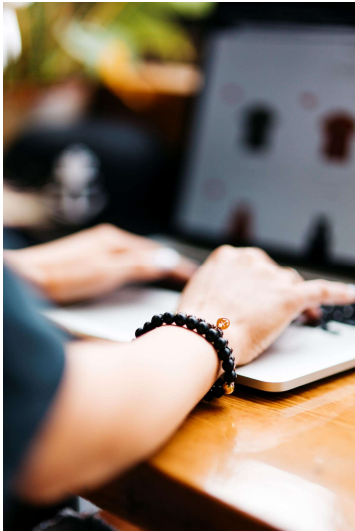




Upper Extremity Cases

PART 2

BRONWYN SING, DO



Case 1

A 32 y.o. female presents to your office with right sided wrist pain. She states the pain started after having her first child 4 months ago. She states the pain is worse when she lifts her baby or holds her baby to breastfeed.

Case 1

What is your differential diagnosis?

What examination would you like to perform?

What are the anatomic structures involved in this condition?

What is FIRST LINE treatment for this condition?

DDx: carpal tunnel, wrist sprain, thumb OA,

Inspection, palpation, ROM. Phalen's, Finkelstein

abductor pollicis longus and extensor pollicis brevis tendons at the wrist

2 week course of NSAIDs and a thumb spica splint that immobilizes both the wrist and thumb

Case 2

A 42 y.o. male presents with right elbow pain for the last three weeks. He works as an electrician. He notices the pain is worse at work. He has tried ibuprofen and it helps the pain a little.

On exam

- Inspection: symmetrical
- Palpation: tenderness to palpation on the right lateral elbow
- What special tests would you perform to help with your diagnosis?

Wrist extension and supination

What anatomic structures are involved in this condition?

Wrist extensors

What imaging is indicated for this patient?

Xrays if ROM or joint bony changes

What is FIRST LINE treatment for this patient?

Modifying or eliminating the activities that cause symptoms is the most important step to treatment

Case 2

What motions do you expect to cause pain on exam?

What anatomic structures are involved in this condition?

Is imaging indicated for this patient?

What is FIRST LINE treatment for this patient?

- Wrist extension and supination
- Wrist extensors
- Xrays if ROM or joint bony changes
- Modifying or eliminating the activities that cause symptoms is the most important step to treatment/ forearm brace

Case 3

A 43 y.o. female with type 2 diabetes presents with right hand numbness, she notes more in the thumb and 2nd digit. She states the pain and numbness wakes her up at night.

PMHx: Obesity, tobacco use

On exam:

- Inspection: symmetrical
- Palpation: nontender at elbow and wrist
- ROM: full active and passive ROM of elbow and wrist

What provocative test(s) do you expect to be positive?

Tinels, Phalens

What risk factors does this patient have?

DM, Gender

What is FIRST LINE treatment?

Splinting wrist, NSAIDS

What study can be done to make a definitive diagnosis?

EMG, remember 5-10% of patients may have normal results

Case 3

What provocative test(s) do you expect to be positive?

If this were a chronic issue- what might you expect to see on exam?

What risk factors does this patient have?

What is FIRST LINE treatment?

What study can be done to make a definitive diagnosis?

- Tinels, Phalens
- Thenar atrophy
- DM, Gender
- Splinting wrist, NSAIDS
- EMG, remember 5-10% of patients may have normal results